

Introduction

There is **fragmented and ineffectual oversight** of AI in Sport & Exercise Medicine (SEM) and adjacent fields including Orthopaedic Surgery, Physiotherapy, Public Health and Primary Care, and performance sport environments.

1 in 7 of UK public use **AI chatbots instead of contacting GP**.

Problematic advice in 50% of responses from AI chatbots.

Voluntary regulation and **low adherence to reporting standards**.

WHO: patient-facing AI tools are clinical-wellness **grey zone**.

Underrepresented patients & non-clinicians in regulation work.

Our recent review (Morgan-Young, in progress) recommended **stakeholder & professional body involvement** in design, research, and governance is a priority for responsible implementation.

Aims

- 1) Evaluate **stakeholder perspectives** across relevant SEM groups & domains;
- 2) Construct a structured, **evidence-based consensus study**.

Methods – Phase One: Cross-sectional Stakeholder Survey **Open now**

Instruments

AIAS-4 for **general attitude** (Grassini 2023)
Extended UTAUT for **acceptance** (Zhai 2021)
Identity **threat & enhancement** (Jussupow 2022)
OAAIMS for **service users** (Kantor 2026)

Bespoke tools evaluating: **autonomy, escalation, governance, disclosure**.

Twelve stakeholder groups

Identified by literature review (Morgan-Young)
Sport & Exercise Medicine, Orthopaedic Surgery, Primary Care, Physiotherapy, Sports Science, Allied Healthcare, Coaching, Product Development, Procurement & Implementation, Corporate & Investment, Regulation, Patients.

Methods – Phase Two: Modified Delphi Consensus **Recruiting now**

Agreement threshold

70% pooled (within-group if $n \geq 20$). Symmetric disagreement carried as negative statements

Platform architecture

Symposium bespoke research platform leveraging open source/commercial services (AJ Hall, St Andrews, UK):

Aletheia – search & appraisal (e.g. NCBI E-utilities, EuropePMC API);

Althing – survey (e.g. survey.js)

Delphi – consensus-building (RAND/UCLA protocol)

Data curated securely (PostgreSQL; Supabase). Analysis (Python; various)

Results –Stakeholder group mapping & two-phase process design



Figure 1. Twelve stakeholder groups against five domains of use.



Figure 2. Two-phase study design: cross-sectional survey, formal consensus.

Invitation to participate

Participate in **Phase 1: Survey**

We are surveying clinicians, allied professionals, patients, coaches, procurement, administrators, policymakers, and others.
Open until March 2027

Register interest in Phase 2: Delphi, the consensus study to convert survey findings into specialty-owned statements for policymakers and practitioners.

Read more online

Complete survey

Register for Delphi



Scan QR code →

Discussion

The **Stakeholder survey** has merit as a **standalone study**:

The first cross-disciplinary, patient-inclusive map of AI perspectives in SEM, capturing groups underrepresented in the standards literature.

Although it sampling **range** but not **prevalence** of opinions, it provides relevant, tested, valid statements to support **formal consensus-building via Delphi**.

Together, the two phases **convert inferred priorities into a consensus** that is **specialty-owned, auditable, evidence-backed, and robust** to guide AI tool users, developers, regulators, and professional bodies.

References

1. King's Health Partners. The use of AI in UK healthcare. KCL; 2026.
2. WHO. Guidance on large multi-modal models. Geneva: WHO; 2024.
3. Grassini S. Front Psychol 2023;14:1191628.
4. Zhai H, et al. J Med Internet Res 2021;23:e27122.
5. Jussupow E, Spohrer K, Heinzl A. JMIR Form Res 2022;6:e28750.
6. Kantor J, et al. JAAD Int 2026;26:113-121.
7. Eysenbach G. J Med Internet Res 2004;6:e34 (CHERRIES).
8. Gattrell WT, et al. PLoS Med 2024;21:e1004326 (ACCORD).
9. Diamond IR, et al. J Clin Epidemiol 2014;67:401-409.